

Stroke Research Program Policy

Aligned with Joint Commission Comprehensive Stroke Center research requirements and AHA/ASA scientific engagement expectations

1. Purpose

This policy establishes the framework for the stroke program's research activities, including governance, human subjects protections, study identification and prioritization, enrollment procedures for time-critical acute stroke research, data integrity, conflict-of-interest management, and reporting — consistent with Joint Commission Comprehensive Stroke Center (CSC) certification requirements for participation in stroke-related research and scholarly activity.

Research program requirements are most directly applicable to Comprehensive Stroke Center and, in some cases, Thrombectomy-Capable Stroke Center certification; confirm applicability to this facility's specific certification level.

2. Governance Structure

- ☐ A Stroke Research Committee (or designated subcommittee of the Stroke Committee) oversees the research program, meeting at a defined interval (e.g., quarterly)
- ☐ Stroke Research Committee reports to the Stroke Center Medical Director and, for study-specific matters, to the Institutional Review Board (IRB)
- ☐ Stroke Research Committee membership includes: Stroke Medical Director (or designee), research coordinator(s), a biostatistician or research methodologist (as available), and representation from nursing/allied health as appropriate
- ☐ All human subjects research is reviewed and approved by the IRB (or ceded to an external/central IRB per institutional reliance agreement) before initiation — no exceptions outside of IRB-approved emergency research provisions (see Section 6)

3. Research Program Scope & Objectives

- ☐ Maintain active participation in stroke-related research, which may include: investigator-initiated studies, multi-center therapeutic/device clinical trials, registry-based outcomes research, quality improvement research, and translational/basic science collaborations
- ☐ Maintain at least one actively enrolling or active stroke-related research protocol at any given time, consistent with certification expectations — confirm current minimum activity threshold against the applicable certification manual
- ☐ Support scholarly dissemination (peer-reviewed publication, conference presentation) of stroke program research and quality outcomes
- ☐ Align research priorities with patient population needs and gaps identified through registry/quality data (see institutional Stroke Registry Policy)

4. Study Identification & Prioritization

- ☐ New research opportunities (investigator-initiated or externally sponsored) submitted to the Stroke Research Committee for initial review
- ☐ Committee evaluates proposed studies for: scientific/clinical relevance to the stroke population served, feasibility (patient volume, staffing, budget), regulatory complexity, and alignment with program priorities
- ☐ Studies recommended for pursuit are routed to the IRB and, where applicable, institutional contracts/budget review before initiation
- ☐ Committee maintains a running log of active, pending, and completed studies for internal tracking and certification reporting

5. Roles & Responsibilities

- ☐ Principal Investigator (PI): physician (or qualified investigator per institutional policy) responsible for scientific conduct, regulatory compliance, and oversight of each study
- ☐ Research Coordinator(s): manage day-to-day study operations, screening, consent process support, data collection, and regulatory documentation
- ☐ Stroke Medical Director: oversees alignment of research activity with clinical program priorities and patient safety
- ☐ IRB: reviews and approves study protocols, informed consent documents, and any required emergency-research provisions; conducts ongoing safety/compliance oversight
- ☐ Sponsor/Data Safety Monitoring Board (for applicable trials): independent safety oversight per study-specific charter

6. Human Subjects Protections in Acute Stroke Research

Acute stroke research frequently involves a narrow treatment window and patients who may lack decisional capacity (due to aphasia, altered consciousness, or cognitive impairment), which raises distinct consent considerations.

- ☐ Standard informed consent obtained directly from the patient whenever the patient retains decisional capacity to provide it
- ☐ Legally authorized representative (LAR)/surrogate consent process used per institutional and IRB policy when the patient lacks capacity, consistent with applicable state law
- ☐ For studies conducted under Exception from Informed Consent for Emergency Research (21 CFR 50.24) or equivalent, the institution follows all required regulatory elements, including community consultation, public disclosure, and independent physician concurrence, as approved by the IRB — no such study proceeds without full IRB and regulatory authorization
- ☐ Consent process, capacity assessment, and surrogate identification documented in the research record for every enrolled subject
- ☐ Vulnerable-subject protections (as applicable) followed per institutional IRB policy
- ☐ **Research screening/enrollment activities do not delay or alter standard, guideline-concordant emergency stroke care (e.g., thrombolysis, thrombectomy) — clinical care decisions take precedence over research protocol procedures at all times**

7. Recruitment, Screening & Enrollment

- ☐ Research coordinator(s) screen eligible patients per study-specific inclusion/exclusion criteria, integrated with — not in place of — standard clinical stroke workflow
- ☐ Clinical care team retains authority over emergent treatment decisions independent of research enrollment status
- ☐ Recruitment conducted without coercion; patients/surrogates informed that participation is voluntary and declining does not affect the standard of clinical care provided
- ☐ Screening logs maintained to document reasons for non-enrollment, supporting study feasibility review and CONSORT-type reporting where applicable

8. Data Management & Integrity

- ☐ Research data collected and stored per study-specific data management plan and institutional data security policy, separate from (but may reference) the clinical stroke registry
- ☐ Source document verification and data quality checks performed per study protocol and institutional research monitoring policy
- ☐ Adverse events and protocol deviations reported to the IRB and sponsor/DSMB per required timelines
- ☐ Data sharing with external sponsors/collaborators governed by applicable data use agreements and institutional legal/compliance review

9. Conflict of Interest Management

- ☐ All investigators and key study personnel complete financial and other conflict-of-interest disclosures per institutional policy
- ☐ Identified conflicts reviewed by the institutional COI committee; management plans implemented as required before study initiation
- ☐ Industry-sponsored study relationships (funding, consulting, equity) disclosed in any resulting publications and presentations per journal/conference requirements

10. Publication & Dissemination

- ☐ Stroke program supports and tracks peer-reviewed publication and scholarly presentation of research and quality improvement findings
- ☐ Authorship determined per institutional policy and standard authorship criteria (e.g., ICMJE guidelines)
- ☐ Publications and presentations arising from the program logged for annual certification reporting

Confirm current minimum publication/scholarly-activity expectations against the applicable Comprehensive Stroke Center certification manual, as numeric thresholds are subject to periodic revision.

11. Resource Support

- ☐ Institution allocates resources to support the research program, which may include protected investigator time, research coordinator staffing, biostatistical support, and study startup/regulatory support
- ☐ Funding sources for research infrastructure and individual studies (institutional, grant, industry-sponsored) tracked and reported per institutional research administration policy

12. Certification & Reporting Requirements

- ☐ Annual (or per certification cycle) summary of research program activity compiled for stroke center certification reporting, including: active/completed protocols, enrollment numbers, publications, and presentations
- ☐ Research program activity reviewed at Stroke Committee meetings and made available for on-site survey/review upon request

Confirm current reporting frequency and required content against the certifying body's current manual for this facility's certification level.

Policy Approval

Role	Signature	Date
Stroke Medical Director		
IRB Chair (or designee)		
Research Program Director/Administrator		

Next Scheduled Policy Review Date: _____

References: The Joint Commission Comprehensive Stroke Center (and applicable Thrombectomy-Capable Stroke Center) certification program manual requirements for research and scholarly activity; AHA/ASA scientific statements and Get With The Guidelines–Stroke program; 21 CFR 50.24 (Exception from Informed Consent for Emergency Research); ICMJE authorship criteria; institutional IRB policies. This template must be re-verified against the current certifying-body manual and applicable federal/state human subjects research regulations at the time of institutional adoption.